PROGRAM OVERVIEW FOR REFERRAL PARTNERS

Spruce Street

Long-term structured recovery for men 24–35
Philadelphia

PRIVATE PAY · TWELVE STEP · PROGRAM IN DEVELOPMENT



PART ONE OF TWO

CONDENSED PROGRAM OVERVIEW

THE CONDENSED OVERVIEW

Spruce Street — Program Overview, condensed

Part One is a condensed referral brief, written to stand on its own. Part Two restates and expands the same program in full, adding the rationale, case examples and operating detail.

Spruce Street

Long-term structured recovery for men 24–35
Philadelphia

www.sprucestreetrecovery.com

Spruce Street is a twelve-month, residential, social model recovery program in development in Philadelphia, expected to open by the end of 2027. It is built for men who have achieved clinical stability but have not yet demonstrated the sustained behavioral change — honesty, follow-through, planning and accepting help — that stabilization alone does not reliably produce.

Spruce Street provides no clinical treatment. Residents retain outside therapists, psychiatrists and physicians, and the house coordinates with them by consent. This summary is written for psychiatric offices, inpatient and residential treatment programs, PHPs/IOPs, detoxes and other referral professionals evaluating placement options for this population. **Part Two restates and expands this same program overview in full beginning on page 11.**

MEN 24–35 · ROUGHLY 18 BEDS · 12–14 MONTHS
PRIVATE PAY · TWELVE STEP · PRE-OPENING

Program at a Glance

POPULATION Men ages 24–35	**LENGTH** ~12 months to graduation; 12–14 including in-house graduation	**CAPACITY** Roughly 18 beds; final count depends on property and approvals
LOCATION Philadelphia; property search underway	**FRAMEWORK** Twelve Step; all Steps completed with a sponsor	**COST** Private pay; expected $5,000–$7,000/month; outside clinical care separate
CLINICAL SCOPE No in-house therapy, psychiatry, medical care or detox	**ADMISSIONS** Director-led assessment; clinician consultation; family interview; voluntary entry	**CERTIFICATION** Intends to pursue PARR/NARR certification

SECTION 1

Clinical Rationale

Relapse is usually preceded by a period of behavioral drift rather than a sudden lapse: an avoided conflict, an incomplete disclosure, a deferred problem, a return to solitary coping — often weeks or months before a substance is used again. These patterns are frequently insight-resistant. A resident may describe his own avoidance or dishonesty accurately and still repeat it under fatigue, embarrassment or inconvenience.

Detox, residential treatment, and outpatient psychiatric and therapeutic care each serve a distinct and necessary function: medical stabilization, and the diagnosis and treatment of underlying conditions. Conventional sober living contributes structure, abstinence monitoring and recovery peers. What this continuum does not reliably supply is **sustained behavioral practice under ordinary, low-stakes conditions** — enough time, proximity and specificity for a maladaptive pattern to recur, be observed, and be met with a different response before it escalates.

Spruce Street is designed to occupy that gap: an extended practice environment, comparable to a flight simulator, in which the behaviors that precede relapse are surfaced deliberately and under supervision, while consequences remain survivable rather than catastrophic.

SECTION 2

Program Model: Reciprocal Responsibility

The program's operating mechanism is peer reciprocity under staff authority — not a service model in which staff act upon passive residents. A newer resident's restrictions (accompaniment requirements, no independent movement, no personal phone, house-held finances) create structured obligations for a senior resident, who must accompany, teach and report without assuming decision-making power. As trust is earned, roles invert: the same resident later carries responsibility for someone newer.

- Residents supply proximity, attention and unfiltered behavioral information that no staffing ratio can reproduce.
- Staff retain all clinical, disciplinary and safety authority. **No resident determines another resident's standing, consequences, money, medication or freedom.**

- Peer closeness is the instrument; institutional authority remains exclusively with staff.

> ILLUSTRATIVE EXAMPLE
>
> A second-month resident requesting money for a haircut must submit the request in writing that morning, arrange a senior resident's accompaniment, and plan around a fixed evening commitment — practicing anticipation, direct asking and tolerance of a possible "no." The same resident, nine months later, faces the transaction from the opposite side: whether to spend his own time teaching a newer resident the process, or simply complete it for him.

SECTION 3

Admissions Criteria

Indicated for men who:

- remain sober within a structured setting but destabilize as structure loosens;
- understand their relapse patterns without having reliably changed the underlying behavior;
- need to rebuild ordinary competence — work, finances, medical or legal follow-through — alongside recovery; and
- can live safely with other residents in a demanding communal setting.

Both first-time and multiple-treatment-episode candidates are appropriate; prior treatment failure is not a disqualifier.

Declined or deferred for:

- acute withdrawal or medical instability;
- psychiatric symptoms that cannot be managed safely in a non-clinical residence;
- active risk of harm to self or others, or a strong recent history of violence; and
- court-ordered residence as a condition of placement (probation/parole reporting can be accommodated where entry itself remains voluntary).

Admission requires voluntary entry, an initial 90-day commitment, and a Director-led assessment that includes consultation with the referring professional and a family interview.

SECTION 4

Program Structure: Phases

Advancement is earned, not scheduled: each increase in freedom is requested by the resident, discussed with peers, and granted by staff based on demonstrated honesty, follow-through and step work. Timings below are typical, not guaranteed.

PHASE	TIMING	KEY FEATURES
Arrival	Week 1	Paired continuously with a senior resident; oriented to house systems
Early Structure	Months 1–3	No independent movement, no personal phone, house-held money, daily accountability groups
Standing	~Month 3+	May accompany newer residents; begins assuming peer responsibility; reversible based on conduct
Employment	~Month 6+	Job search and part-time work (30–35 hrs/wk); house obligations continue unchanged
Senior Leadership	Final quarter	Orients new arrivals; exercises judgment without institutional authority
In-House Graduation	~Month 12+	Phone/laptop return; independent movement and dating resume while still residing in house
Graduation	~Months 12–14	Staff-determined on demonstrated reliability, not time elapsed

SECTION 5

Accountability System

- **Daily structure** includes a brief morning operations group and a required 4:30 PM "House Business" meeting (peer feedback, Project checks, Standing requests).
- **Projects** translate a targeted behavior into a specific, observable, time-limited action tied to an actual pattern — not a general aspiration (e.g., "be more honest"). Two tests govern every Project: an observer can tell whether it happened, and it targets what the resident *does*, not who he *is*.
- Each Project is monitored daily by an assigned peer "checker." Staff assign every Project and make the final call.
- Written reflection follows each House Business meeting; a weekly "Big Group" addresses longer-standing patterns.
- Testing is observed, full-panel, random and scheduled, with laboratory confirmation of anomalous results.

Representative daily schedule

7:30	Wake; breakfast, gym, step work, written money requests
9:00	Medication and money window
9:30	Chores
11:00	Morning group
12:00–4:30	Appointments, sponsor, Projects, job search, errands
4:30	"House Business" meeting
6:30–8:30	Outside Twelve Step meetings, attended in groups
8:30–11:00	Written reflection; free time

SECTION 6

Clinical Boundaries & Safety

CLINICAL CARE	**No clinical treatment is provided.** Therapy, psychiatry, medical care and detox remain outside the house; outside providers retain full clinical authority.
MEDICATION	Prescribed and managed externally; stored in a locked staff office and self-administered at scheduled windows with staff present. No in-house prescribing, adjustment or taper direction.
OPIOID MAINTENANCE	Residents may enter on maintenance therapy. Program expectation is completion without it; the outside prescriber controls the pace and clinical appropriateness of any taper.
RELAPSE	**Not automatic discharge.** Typical response is detox with possible return, or continued residence under restricted status pending assessment; no resident is discharged to the street for a relapse.
EMERGENCIES	Naloxone is stocked house-wide; staff and residents are trained. Awake staff remain on site overnight. **No one is ever sanctioned for calling for emergency help.**
VOLUNTARY RESIDENCY	Staff do not restrict departure or withhold belongings. Court-ordered residence is not accepted; probation/parole reporting can be accommodated when entry remains voluntary.
CONFIDENTIALITY	Written commitment among residents. Disclosure to families or clinicians occurs under consent, or when safety or legal obligations require otherwise.
PEER AUTHORITY	**Observational and supportive only.** Peers never hold disciplinary, financial, medication or safety authority; that responsibility remains exclusively with staff.

> ***Model lineage.*** *The program draws on therapeutic-community practice — peer accountability, restricted movement and structured group process — while explicitly excluding punitive or humiliation-based practices. Discomfort is permitted; degradation is not.*

SECTION 7

Family and Referral Partner Coordination

- Family communication is generally weekly during the first months (with a signed release) and as needed afterward; in-person family gatherings occur every two weeks, hosted by residents.
- With consent, referring clinicians receive a behavioral second opinion — how a resident's stated insight holds up under real conditions (work, conflict, money, unstructured time). Communication is intended to be useful and proportionate, not a stream of minor infractions.
- The program requests reciprocal candor from referring professionals: relapse precursors observed, clinical needs the house cannot meet, and any concerns about fit or safety.
- When a placement is a poor fit or a resident requires a higher level of care, Spruce Street will say so directly rather than defend the placement.

SECTION 8

Outcomes Measurement and Development Status

Spruce Street has not yet admitted residents and has no outcomes of its own. From first admission, the program intends to track: length of stay (including early departures); phase reached and graduation status; destination and level of care after departure; and sobriety, housing, employment/education and recovery engagement at 12 and 24 months — reported alongside the response rate achieved at each interval.

Settled: population, program length, phase structure, accountability system, Twelve Step foundation, private-pay structure, non-clinical scope, and opioid-maintenance policy.

In progress: property and municipal approval, final capacity and fee, certification timing, admissions and medication policies, staffing ratios, and a clinical/operator advisory circle. Target opening: end of 2027.

Founders

Ty Stillerman served four years as Assistant Program Director of New Life House (Los Angeles), the program on which this model is based. He holds a B.A. in Economics and Public Policy from UC Berkeley and will direct Spruce Street's programming and daily operations. **Chase Wilson** spent three years as a Program Manager at The People Concern (Los Angeles), leading outreach connecting unhoused individuals to treatment and housing. He holds a Master's in Urban Planning from the University of Southern California, where he is an MBA candidate expected to graduate in December 2026, and will lead business operations and finance.

We are circulating this overview before opening to build referral relationships around this population's actual standards. We welcome direct input on what you would need to see before referring someone — certification, written policy, clinical relationships, staff qualifications, facility standards, references or something else. Uncomfortable feedback is welcome.

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This document describes a program in development and is not an offer of services.

END OF CONDENSED OVERVIEW

Part Two restates and expands this same overview for readers who want the full rationale, examples and operating detail.



PART TWO OF TWO

FULL PROGRAM OVERVIEW

THE FULL OVERVIEW

Spruce Street — Program Overview, unabridged

Part Two restates and expands the same program described in Part One. Part One is designed to stand on its own as a quick referral brief; what follows adds the full rationale, case examples and operating detail.

Spruce Street

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Philadelphia

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Spruce Street is a twelve-month, residential, social model recovery program in development in Philadelphia, expected to open by the end of 2027.

This overview is written for clinicians, treatment programs, interventionists and others who help families decide what should follow stabilization. Its purpose is to make the model concrete enough to evaluate: what the house does, why its structure exists, and the limitations in our scope of care.

We are sharing this overview before opening because we want to build toward the needs and preferences of people who refer and treat this population. We welcome direct feedback, and are especially interested in **what you would need to see before referring someone — certification, written policies, clinical relationships, staff qualifications, facility standards, references or something else?**

Uncomfortable feedback is welcome. A program built around observable conduct should be prepared to have its own conduct examined.

MEN 24–35 · ROUGHLY 18 BEDS · 12–14 MONTHS
PRIVATE PAY · TWELVE STEP

> *Spruce Street has not yet opened and has no outcomes of its own. The model draws on Ty Stillerman's four years operating the program on which it is based, both founders' experience as residents there, and their later work in recovery and social services. That experience supports building Spruce Street; it is not evidence of Spruce Street's results.*

SUMMARY

The Model in Brief

Spruce Street is built for men whose recovery has to change how they live, not merely remove them from the drink or the drug. Stabilization, therapy and psychiatry are often essential. But a man may be sober, insightful and clinically stable while the behaviors that precede relapse — avoidance, dishonesty, isolation, impulsivity and broken commitments — remain intact. Honesty, planning and follow-through cannot be conferred in an appointment. They have to be practiced under real obligations to other people, where conduct is visible, correction is immediate, and mistakes matter without becoming catastrophic.

Spruce Street holds that practice environment for a year. Residents live together, attend outside Twelve Step meetings, work with sponsors, keep the house running, find jobs and gradually resume independent life. The individual ingredients are familiar. What distinguishes the model is how deliberately they overlap. Small obligations fill the day and expose patterns early — before a manageable failure becomes another crisis.

The structure is also reciprocal. A new resident who cannot travel alone is protected; his dependence gives a senior resident an hour in which he must be patient, attentive and reliable. A resident who needs help arranging a ride has to plan, ask and tolerate hearing no; the person helping him has to serve without taking over. Nearly every restriction placed on one man creates useful responsibility for another. Both are observed. Both receive feedback.

Residents provide proximity, attention and honest information. Staff hold authority. Residents accompany, teach, check, notice and say what they see. They do not decide another resident's standing, consequences, money, medication or freedom. Staff run every group, make those decisions and remain responsible for safety.

As trust grows, restriction gives way to responsibility. Work, money, technology, independent movement, private plans and dating return in sequence while the resident is still surrounded by people who know his patterns. The aim is not a year of good behavior inside a controlled environment. It is self-governance as the controls come away.

Spruce Street provides no clinical treatment. Residents use outside therapists, psychiatrists and physicians, and the house coordinates with them by consent. The program is grounded in the Twelve Steps, worked in full with a sponsor before graduation. Residents may enter on opioid-maintenance medication, with the expectation that they will be off it by graduation under the direction of their outside prescriber.

PART TWO

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SECTION 1

The Gap: What Happens Between Treatment and Independence

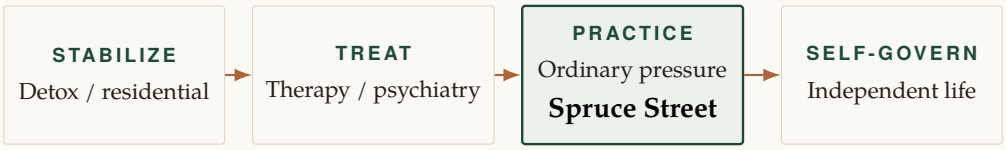
Relapse rarely begins with a craving. The drink or the drug is usually the last domino, not the first.

The sequence often starts weeks or months earlier, in ordinary behavior. A conflict is avoided until it hardens into resentment. A family hears most of the truth and the missing piece does its work. A manageable problem sits untouched until it becomes a crisis. A bad stretch gets handled the old way — alone, behind a closed door. By the time the substance returns, the life around it may already have been reorganizing for some time.

Those behaviors are difficult to change from a distance. Insight matters, but it does not reliably survive embarrassment, fatigue, inconvenience or fear. Someone can understand exactly why he avoids asking for help and avoid it again the next time he needs it. He can recognize his dishonesty and still tell the easier version because the truth would make him look foolish. **Understanding often arrives long before conduct changes, and the interval is where much of the danger lives.**

For some men the result is a familiar cycle: stabilize, step down, drift, return. He does well while something is holding him; independence arrives before the behavior has changed, and the old pattern reassembles itself quietly, in private, until the fall is already underway. Each round costs the family another season of fear and grief. It also hands him one more piece of evidence for the conclusion he may already be reaching: that he is the exception, and there is no solution for him.

Serious addiction usually requires several kinds of help, each with a distinct purpose. Detox and residential treatment arrest the harm and create safety. Therapy and psychiatry identify and treat what sits underneath. Conventional sober living can provide a sober address, clear expectations, recovery peers and distance from the setting where someone was using. For many people, that sequence is enough. All of it is real work.



What it does not always provide is sustained behavioral practice under ordinary pressure: enough time for patterns to recur, enough proximity for them to be seen, and enough specificity for a different response to be tried the next day — in the kitchen, during an empty afternoon, in a conflict at work, and on the day he is ashamed of what he has done.

That function often has no clear owner. A therapist has an hour and works largely from the client's account of the other 167. A residential program has a defined episode of care. A sober house may enforce abstinence, curfew, meetings and employment without being designed to see what is happening beneath visible compliance. None should be faulted for falling short of a function they were not designed to provide. Part of the reason is economic. The work is slow, repetitive and largely unbillable — there is no code for a man learning to tell the truth when a lie would serve him. That is why families pay for this house directly, and why we would rather call it what it is than describe it as treatment.

The structural problem is that a resident can attend group, go to work, pass the test and return to a room where nobody sees that he has stopped calling his sponsor, begun avoiding a housemate or told his parents a version of the week that is not quite true. Someone who has been through treatment several times may be especially fluent in the visible parts. He can describe his patterns accurately, agree with every expectation and sincerely want to change while still concealing small failures, manufacturing emergencies or withdrawing when ashamed.

Nobody has necessarily done anything wrong. The clinician stayed within scope. The treatment program delivered what it promised. The sober living enforced its rules. The family respected the boundaries it was given. The resident complied. And the pattern that brought him there remained intact.

Spruce Street is designed to own that practice environment. It extends treatment through a year of ordinary life under unusually close structure. The closest analogy is a flight simulator: difficult conditions encountered deliberately and in sequence, where mistakes are survivable and someone watches closely enough to see where things went off course.

The house closes many of the private spaces where old behavior lives, then fills the day with ordinary demands that cannot be handled entirely through performance:

a commitment kept, a request made in advance, a chore passed inspection, a ride arranged before it becomes urgent, a difficult thing said to the person it concerns. The expectations are numerous so each can remain small. A forgotten appointment or neglected chore is not a grave matter; it is information about what the resident does when disorganization, correction or honesty causes discomfort. The mundane is also difficult to perform for effect: a resident can sustain a polished account of himself for an hour in an office, but not across dozens of overlapping obligations, in front of seventeen other men, for a year. Eventually a plan changes, a request is refused or he becomes tired enough to do what he would do unattended. That is when the useful information arrives.

The purpose is not compliance. A tightly controlled resident can look stable without becoming free. The structure must therefore loosen while the resident remains observed: work, money, technology, movement and privacy returned one layer at a time. The relevant question is not whether he can follow the rules of a sober house. It is whether he can govern himself as they fall away.

That requires more than staff supervision. It requires a community whose members carry real obligations to one another, so that receiving help and becoming useful are part of the same progression. That is the operating center of Spruce Street.

SECTION 2

The Method: Reciprocal Responsibility

Spruce Street is not a set of services delivered by staff to residents. It is a community arranged so that one resident's temporary dependence becomes another resident's useful work. The same ordinary situation asks something different of each man, and something different of the same man as his role changes over the year.

Two terms help make the following example concrete. A **senior resident** is far enough along to be trusted with someone newer; in the early months, a resident goes nowhere without one. The **ride sheet** lists people who may be called for transportation to meetings or appointments: residents with cars, friends of the recovery community and, in time, alumni.

Consider a haircut.

ONE ERRAND

A resident in his second month decides he wants a haircut. The money must be requested in writing that morning, which means anticipating the want rather than acting on it at the moment it becomes urgent. He needs a senior resident willing to accompany him, so the errand has to fit another person's day. He must be back for the 4:30 "House Business" meeting. And before the haircut happens, he needs a credible plan for that evening's required Twelve Step meeting.

In his second month, he cannot make the ride calls alone. A senior resident sits beside him while he works the ride sheet: calling people whose evenings do not revolve around him, hearing no, tolerating the awkwardness and trying the next number. If a ride is not secured in time, the haircut waits.

The restrictions protect him, but protection is only half their purpose. He is practicing anticipation, asking directly, accepting dependence and arranging a desire around commitments that already exist. Whether his adult strategy has been to need nobody or to need everybody at once, the asking is the work.

The senior resident has work of his own. He must be patient without taking over, keep track of someone else's money and make sure both residents meet their obligations. If the newer man procrastinates or turns poor planning into an emergency, the senior resident must notice and say so. If he becomes controlling, performs generosity or quietly does the task himself, that becomes visible too.

Nine months later, our newcomer has become a senior resident himself, and decides he wants another haircut. He can arrange it alone, but despite his plans, a newer resident needs his assistance making ride calls. Teaching this man — who sits exactly where he did just months ago — will take longer than making the calls himself, and may leave no time for the haircut. Now, our senior resident faces the other side of the exchange: whether he uses the competence he has gained only to make his own life easier or spends some of it helping another man learn.

None of this is really about hair. Early in the year, what he wants has to bend around dependence on somebody else's time. Later, the same want has to bend around responsibility to somebody newer. In both cases, the errand puts desire in contact with obligation and makes visible which one gives. That is reciprocal responsibility.

The same shape repeats throughout the house. A resident who needs a ride gives someone else the experience of being depended upon and coming through. A man assigned a daily Project gives his checker somebody's work to remember besides his own. A new arrival who is frightened and exhausting gives a senior resident a week in which to practice patience and grace.

Observation runs in both directions. Staff may discover that the person assigned to inspect chores has been waving them through to avoid conflict, or that a resident offering feedback enjoys the authority a little too much. Responsibility does not lift anyone above scrutiny. It places more of his behavior where other people can see it.

Accountability, community and structure are the field's three most overused words. Accountability here is not a few house norms and an occasional conversation; it is many small expectations, enforced immediately and predictably, so that falling short produces routine information rather than a crisis. Community here means more than sober people sharing an address, and structure means more than curfew, testing and a meeting schedule. Residents need one another for accompaniment, rides, instruction, honest feedback and company. Daily life makes planning, honesty, reliability, service and accepting help into repeated obligations. Nearly every act of receiving creates work for the person providing it.

The division of labor is deliberate. Residents supply a density of attention that no reasonable staffing plan can reproduce in the same form. They live beside one another. They know who has gone quiet, who is avoiding the phone, who is privately angry, who is performing engagement and who is beginning to disappear. They accompany, check, teach, ask and report what they see.

Staff decide what that information means. A resident never determines another resident's standing, consequence, money, medication or freedom. Staff assign Projects, run groups, make decisions and hold responsibility for safety. The model depends on peer proximity; it does not confer institutional power on peers.

The arrangement gives the year its moral arc. A resident arrives needing to be carried through a period when he cannot govern his own day. He becomes dependable, then responsible for someone newer and more frightened. In that movement — from being carried to carrying another — he acquires evidence that he can be useful, trusted and resilient. Self-respect returns in pieces, through small acts worth respecting, until the evidence outweighs the case he has built against himself.

This structure is demanding, but it is not built on enthusiasm. A man can arrive angry, skeptical and certain that the rules are beneath him. What matters at admission is whether he can enter voluntarily, live safely in the community, and make enough of a commitment for the process to begin.

SECTION 3

Admissions: Who the House Is For

Spruce Street is for men ages twenty-four to thirty-five whose recovery requires a sustained change in how they live, not only separation from a substance.

They arrive by different routes. Some have years of treatment behind them and can describe their patterns fluently. Others are making a first serious attempt after detox, residential care or a collapse of another kind. **Nobody has to fail repeatedly before he deserves a year of real support.** The first attempt may be the moment to keep the cycle from taking hold.

The fit tends to be a man who:

- can remain sober inside a structure but has not remained stable when it loosens;
- may understand his patterns without having reliably changed them;
- struggles with honesty, follow-through, isolation, impulsivity, planning or accepting help;
- needs to rebuild ordinary competence — work, money, cooking, medical care or legal obligations — alongside recovery; and
- can live safely with other people in a demanding communal setting.

The program is grounded in the Twelve Steps. Residents work all twelve in full with a sponsor before graduation. We regard the Steps as a spiritual undertaking rather than a religious one. Nobody is required to profess a belief, and what a resident does with the framework after leaving is his decision and holds no bearing on his standing in the community. During the program, however, the practices underneath it — honesty, humility, service, responsibility and repair — are not optional. They are the principles on which the house operates.

Enthusiasm is not an admission requirement. The model is built to work with men who arrive angry, skeptical or privately convinced that the program is absurd. Willingness often develops after a resident has watched people like him change and has accumulated evidence from his own conduct. A man who comes only because the alternatives have run out is not a poor candidate. He is close to the profile.

Lack of enthusiasm is not lack of consent. Admission is voluntary. A resident must understand the essential structure, choose to enter and make an initial ninety-day commitment. That is not presented as the likely length of the work. It is time to gain stability, experience the program rather than react to its description, and make an informed decision about the year. He remains free to leave at any time.

Spruce Street is not detox, residential treatment, psychiatric care or medical care. Admission is declined or deferred when someone is in acute withdrawal, medically unstable, experiencing psychiatric symptoms that cannot be managed safely in a non-clinical residence, or presenting an active risk of harm to himself or others. We reserve the right to decline a candidate with a strong recent history of violence. A resident must be able to use outside clinicians for the care he needs and live safely beside the other men in the house.

Residents involved with probation or parole may be admitted, and the house will provide required reporting, provided the choice to enter remains voluntary. We do not accept placements in which residence at Spruce Street is itself a court-ordered condition.

Residents may enter while taking buprenorphine, methadone or another opioid-maintenance medication, with the expectation that they will complete the program without it. This must be discussed during admission so that the resident, family, prescriber and house understand the boundary before anyone commits. The outside prescriber controls every clinical decision and any taper. The full division of authority appears in Section 6.

Admissions are led by the Director and include the addiction and treatment history, behavioral fit, informed willingness, safety, current medication and clinical needs; consultation with the referring professional or program; and a family interview. The purpose is not to identify a resident who already knows how to succeed here. It is to determine whether the house can safely create the conditions in which his actual patterns can surface and change.

SECTION 4

The Program Year: From Structure to Self-Governance

The structure incrementally loosens as a resident becomes capable of carrying what replaces it. Progress is not automatic or determined solely by time. Each increase in freedom is requested by the resident, discussed in front of his peers and granted by staff based on honesty, follow-through, step work and the trust he has actually earned. The timings below are typical, not promised. At every stage, what a resident receives changes alongside what he owes.



Restriction gives way to responsibility; freedom returns while conduct is still visible.

Arrival: paired at all times

During the first week, a new resident is paired with a senior resident who goes everywhere with him. Not checks in on him — goes with him: to meetings, appointments and the kitchen at midnight. The pairing reflects both who might reach the newcomer and what the senior resident needs to learn. One man is carried through a period when being alone may be dangerous; the other must remain patient and useful toward someone frightened, angry and often exhausting. One is being kept safe. The other is being taught grace. That is the program in miniature, and it starts on day one. At launch, staff and selected members of the recovery community will fill this function until residents earn senior roles.

The first months: being carried

There is no job yet. The days are outside meetings, house groups, chores, a sponsor, the first Steps, Projects, and attention to the legal, financial and medical wreckage addiction tends to leave behind.

The resident goes nowhere alone — not to a store, a haircut or dinner with his parents. He has no smartphone. Calls home and to his sponsor are made on a house line with a senior resident present. Money is held by the house, requested in writing and carried by the person accompanying him.

Together, those restrictions close most of the spaces in which relapse is planned. In the first months, drinking or using would mean walking out the front door in daylight, past people who would tell him exactly what he is doing. The same restrictions force the resident to anticipate what he needs, ask directly, accept that the answer may be no and arrange his desires around existing obligations. The structure reveals whether he plans or manufactures emergencies, follows through when commitments become inconvenient, and tells the truth about what nobody would otherwise catch. It also shows whether the senior resident keeps his word, resists taking over and speaks when the newcomer begins to withdraw.

Standing: learning to carry

Around the third month, a resident may ask the house for **Standing**. If granted, he stops being only the person accompanied and becomes someone trusted to accompany another. He may take a newer resident to a store, meeting or appointment, sit in on another resident's call home, and bring a newer man along when his own family takes him to dinner.

He receives movement, trust and usefulness. In return, he owes attention to needs that rarely arrive conveniently. He is expected to begin finding the work: noticing who went quiet after a meeting, who has been immobilized by fear, or when the house needs someone to organize the afternoon.

Standing can be reduced when behavior shows it was granted too early. Staff explain why and what rebuilding trust requires. Freedom reflects current evidence of reliability, not a judgment of worth. Earning Standing back often teaches more than holding it did.

Work: the same patterns in work clothes

Around month six, progress permitting, the resident finds a job. The house does not place him. Walking into businesses, hearing no and returning the next day are part of the work. The typical target is a practical "get-well job" of thirty to thirty-five hours a week unless finances require more.

Employment reopens the world. A boss, paycheck, exhaustion, boredom and ambition return while money is still managed, evenings still end at the house, and the people around him know his patterns. The job does not suspend house obligations: a chore must be covered, outside meetings continue, and how early he asks for help remains visible. When poor planning makes him late, it is the transportation problem from month two, wearing work clothes.

The senior months: becoming part of the structure

By the final quarter, a resident should be one of the people on whom the house depends. He orients new arrivals, drives residents to meetings, checks on the man having a hard afternoon and, on assigned evenings, helps staff manage the flow of the house. He exercises judgment without acquiring authority over another resident. For most of these men, nobody has depended on their judgment for anything in a long time.

Senior residents often see what staff cannot: who disconnects when group ends, performs engagement, nurses a resentment or finds subtle ways to be alone. Their responsibility is to act on what they see. Their own failures remain visible too. The resident who enjoys his role for the wrong reasons, ducks a confrontation or becomes

controlling in the name of helping must face that behavior. Leadership moves him into a more demanding form of accountability.

Building the life afterward

The structure is temporary. During the later months, the resident builds a credible plan for housing, work or education, finances, medical care and continued recovery. Old debts, unfiled taxes, missing documents, a budget and savings matter because they determine whether the next chapter begins on solid ground or on problems waiting to become emergencies.

Larger ambitions are made specific enough to test. "I want to work with my hands" becomes conversations with tradespeople, a comparison of apprenticeship programs and an application. "I may return to school" becomes programs, financing and deadlines. Vague plans sound sincere while postponing contact with reality. A plan written down can be argued with; a missed deadline can be noticed.

Families and residents often want to focus on these matters first because it *looks* like a life being rebuilt. Spruce Street takes it seriously, but not as a substitute for the foundation beneath it. A degree or career cannot carry someone who still disappears when ashamed or abandons commitments under stress. Put a good plan on a foundation of reliable conduct and it can stand. Put it on nothing and it rarely holds.

In-house graduation: freedom while still known

Around month twelve, graduation begins with a final period inside the house rather than an immediate move. The resident's smartphone and laptop return. He moves independently, manages private plans and may begin dating while still returning each night to people who know him well.

The sequencing is deliberate. A resident may have built genuine reliability at work and inside the house without yet encountering the phone, privacy, dating or unstructured Saturdays where he historically became secretive. Spruce Street would rather he meet those conditions while his behavior is still visible. Does the phone consume his attention? Does independence become absence? Do private plans displace recovery commitments?

Earlier, the structure enforced disclosure. Now he must volunteer it: asking for feedback before a problem announces itself and choosing to stay known when nobody makes him. When he moves out a month or two later, the aim is that leaving no longer means escape. He is not fleeing the house; he is carrying it with him.

Graduation is a staff judgment, made with stated reasons and discussed with the resident and family. The program also has an end. We do not hold residents indefinitely because they have become comfortable. If someone leaves with serious concerns, we

say so and recommend what should follow rather than pretending the answer is always more time.

SECTION 5

Accountability in Practice

At Spruce Street, accountability is a daily loop: behavior surfaces, gets named, becomes specific work, gets checked and changes through repetition. The loop depends on several parts working together.

THE DAILY LOOP



The honesty engine

The house maintains many ordinary standards: make the bed, clean what you use, be where you said you would be, call your sponsor, return an item, complete the assigned task. Falling short brings a small and predictable response, usually written reflection that evening. **The purpose is never the rule itself.** It is the moment the rule creates: an ordinary occasion to do the right thing when nobody might notice if he didn't.

The stakes are modest because the system is built less to catch a resident than to let him come forward. He realizes he left the light on and nobody saw it, or that something he told his mother was not quite true. He can report it or keep it. Across dozens of low-stakes decisions, he practices telling the truth when forty-five minutes of writing is the likely cost, until honesty has a chance to hold when the cost is larger.

When something small surfaces later, the conversation is about the difference between being caught and coming forward — between compliance and integrity. If nobody discovers it, he still carries the knowledge that he got away with something. This time it cost little and harmed no one, so he can notice the weight without the wreckage. Many residents have not gone to bed with a genuinely clean conscience in years. The structure lets them remember what that feels like.

When a resident appears to be carrying more than he is saying, staff may ask him to inventory every corner cut, grudge nursed, quiet dishonesty and repair avoided.

Like a Fourth and Fifth Step, the exercise removes concealed material around which a separate life can begin to form.

Projects: behavior made specific

The house does not ask someone simply to "be more honest" or "take responsibility." Aspirations that broad cannot be practiced or verified. The issues that surface in daily behaviors become a **Project**: a defined action, performed for a defined period, aimed at the behavior actually in front of him.

A resident afraid to speak must share at a meeting each night. One who engages with nobody must bring a piece of house business to group, requiring him to notice other people. Someone who avoids conflict raises the concern directly that day. Someone generous only when he receives credit must do one useful thing daily and tell nobody except the peer checking the work.

Some Projects restrict rather than prescribe. Two residents who inflame one another may spend a period on non-communication. Someone using the gym to escape every relationship may lose it temporarily. A man who deflects with humor may be asked not to tell jokes for a week.

Two tests govern the assignment. Someone should be able to tell whether it happened. And it must target what the resident does, not what he is. A Project may create embarrassment, frustration or dependence; it may not work through degradation.

Projects change with the behavior. A resident may complete an action while evading its purpose; the man who learns to plan may begin using plans to control everyone around him. The assignment then changes. Staff assign Projects; residents make the practice observable.

The daily check

Every resident carrying a Project has another resident assigned to check it each day. The questions are concrete: What were you supposed to do? Did the situation arise? What did you do? Where did you cut a corner?

The checker must remember somebody else's work and continue asking when letting it go would be easier. The resident inspecting chores has to enter the bathroom and be frank when the cleaning is subpar. If he waves people through to avoid friction, that becomes the next conversation about him. No role in the house involves only receiving.

The groups

The morning group is short and practical: chores, schedules, transportation and whatever the house needs fixed.

The **"House Business" Meeting**, held at 4:30 each day, is the center of the house. It begins with logistics, Project checks and requests for Standing, then turns to what resident issues have surfaced in the last day. A resident may raise a concern about another. Staff identify the principle at stake, keep the discussion specific and open the floor. Other residents speak directly about what they'd noticed in him. He listens without interrupting.

Sometimes listening is the entire exercise: remaining in the room while people offer an unwelcome account of his behavior and discovering that he can survive it. No resident must accept an account he believes is false, and disagreement is not an offense. He is expected to look first for what may be true and to bring objections back to the room rather than campaign around the house afterward.

The residents offering feedback are working too. Each must decide whether his concern is real or merely irritation and whether he can say something difficult without enjoying it. Quiet residents may be called on because speaking is what they avoid; loudness is not treated as accuracy. Staff stop anything that turns personal or gratuitous.

Once a week, a longer "Big Group" looks beyond the events of the day: the pattern under the pattern, the subject someone has carried for years, or the question of whether a resident is participating honestly or performing. Alumni return, and the house marks sobriety anniversaries together, with cake.

Those groups create the trust on which the rest of the structure depends. A resident may eventually say the thing he believes will change how everyone sees him, and watch for the flinch as he says it. Nobody moves, and nobody rushes to fix it. Another man says he has done something close enough to understand. Someone asks — not about the event — but how long he has been holding it. Afterward, he is no longer merely a person with a secret; he is a person the house truly knows.

Reflection and consequences

Written reflection follows the "House Business" meeting. A peer assigns the topic and reads the result; a lazy or punitive assignment becomes a matter for staff. The purpose is to make the day thinkable rather than merely survived.

When something larger damages trust, the response generates work: an honest account, direct repair, a Project aimed at the pattern and sometimes reduced Standing. Tightening the structure is not a verdict; it acknowledges that the evidence has

changed. The question is what the failure reveals and what practice might produce a different response.

ONE LOOP, START TO FINISH

A resident needs to attend a 6:30 AA meeting that evening and has not arranged a ride. In the past, he would have texted someone at 6:15, skipped the meeting or claimed to feel sick. Here he has no car and, early on, no personal phone. He needed to start in the afternoon, ask residents with cars and call the ride sheet. Instead, at 6:00 he is asking three people to rearrange their evenings.

At the "House Business" meeting, the situation is not treated as a transportation problem. He avoids planning, assumes someone will absorb the consequences and converts his inaction into everyone else's urgency. Residents recognize the dentist appointment from the prior week. Someone notices the more revealing point: he never told anyone he was struggling to make the calls.

Staff assign a Secretary Project: the next day's schedule and rides must be written and confirmed by 4:00 each afternoon, checked by a peer. That night he writes about when he first knew he needed the ride, why he waited, what he expected other people to do and what the delay cost them.

His checker now has work too. He must remember to ask before 4:00, decide whether the plan is real and resist making the calls himself. If the checker completes the task for him, the plan may succeed and the program will fail.

The next time the resident fails to prepare, the house may decline to rescue him. He may miss the meeting — a real consequence and a survivable one, precisely the kind the structure exists to let him have.

The daily rhythm

The operating day gives this loop a predictable container:

7:30	Wake; breakfast, gym, step work and written money requests
9:00	Medication and money window
9:30	Chores
11:00	Morning group

12:00–4:30	Appointments, sponsor, errands, Projects, vocational work, job search, outings and ride planning
4:30	“House Business” meeting
6:30–8:30	Outside meetings, attended in groups
8:30–11:00	Written reflection, then free time

The schedule flexes once a resident is working. The elements do not.

The open middle of the day is deliberate. Independent life contains unstructured time; these hours reveal whether a resident can begin what needs beginning, rest without disappearing, plan the evening and notice that someone else needs him.

The house also plays: hikes, basketball, camping, movies and dinners someone turns into an event because the house needs one. Sometimes recreation exposes behavior that appears nowhere else. More often it is simply evidence that sober life becomes larger and warmer rather than smaller. That does not need to be converted into a lesson.

SECTION 6

Authority, Clinical Boundaries and Safety

A reader familiar with the field will recognize the lineage of this model. Peer accountability, restricted movement, managed money, limited technology and daily group work about behavior come from the therapeutic-community tradition. That tradition has produced real harm alongside its results: humiliation presented as method, group process used to break people down, peers given unchecked power and communities that made leaving feel impossible. We would rather name that ourselves than leave you wondering whether we know.

Spruce Street keeps the specificity, relationships and behavioral practice. It rejects yelling, escalating punishment, ritual humiliation and peer authority. Because the structure is intensive, its limits must be as concrete as its demands.

> *"I ran a house in that tradition for four years. The parts that worked were the relationships and the specificity: knowing a man well enough to name exactly what he was doing and giving him something concrete to do instead. The yelling and escalating punishments mostly produced compliance and resentment. We are keeping the first part."*
>
> — TY STILLERMAN

Residency is voluntary

A resident may leave at any time. Staff will tell him honestly when departure appears to repeat a familiar pattern. They will not block the door, withhold belongings or make leaving difficult. The house helps arrange a safe destination where possible.

Admission is voluntary as well. Probation and parole reporting may be accommodated, but Spruce Street does not accept a placement when residence in the program is itself ordered by a court.

Peers observe; staff decide

Residents notice one another, speak directly, check assigned work, accompany newer men and raise concerns. No resident decides another resident's Standing, consequence, Project, money, medication or freedom. Assignments come from staff. Staff run every group, give reasons for decisions and define the path back when trust has narrowed.

Feedback outside group is expected, but it answers to the same standard: measured, specific and focused on behavior. Peer closeness is the instrument. Institutional power remains with staff.

Discomfort is permitted; degradation is not

The program asks residents to tolerate frustration, embarrassment, dependence and correction. A resident may redo a chore, make an apology without managing the response, lose access to something he has used to avoid people or accept reduced Standing after damaging trust. Addiction asked a great deal of him — persistence, concealment, endurance long after the life around him began to collapse. It is not unreasonable for recovery to ask something difficult in return.

The boundary lies in what the assignment targets and how it works. It may address something a resident is doing and create the discomfort required to practice a different response. It may not attack his standing as a person or make contempt the mechanism. No yelling, personal attacks, ritual humiliation or pile-ons. Staff stop feedback that becomes gratuitous.

Confidentiality and complaints have weight

The model cannot work unless a resident can say something true in a room and trust that it will not travel outside it. Residents commit to confidentiality in writing, and breaches are treated seriously.

Information is shared with families and clinicians under appropriate consent or when law and immediate safety require otherwise. A release is not permission to provide a transcript of the resident's week. Communication supports care and gives an honest account of the larger arc.

A resident may bring a complaint about staff, a consequence or another resident to the Director. An outside point of contact ensures that a concern does not depend on the goodwill of the person it involves. Complaint procedures, non-retaliation rules and mandatory-reporting obligations will be provided at admission.

Spruce Street is not clinical treatment

Spruce Street provides no therapy, psychiatry, medical care or detox. Residents use outside therapists, psychiatrists, physicians and other providers selected and paid for by them or their families. The house holds no financial interest in those choices. Its role is to help the relationship function: transportation, scheduling, follow-through and, with the resident's consent, useful communication.

The two views can inform one another without becoming confused. A clinician may learn how a pattern behaves outside the office; the house may learn what to watch for. The clinician retains clinical judgment. Spruce Street holds the daily behavioral environment. Familiarity does not create a license to diagnose or treat.

Medication

Medication is prescribed and managed by outside providers, stored in a locked staff office and self-administered at scheduled windows with staff present. Spruce Street does not diagnose, prescribe, adjust, withhold or advise on medication. Residents take what their outside prescribers direct.

The program has a specific boundary regarding opioid-maintenance medication. A resident may enter while taking buprenorphine, methadone or another maintenance medication, but Spruce Street is designed for him to complete the program without it. Any taper must occur under the direction of the outside prescriber, who controls its pace and determines what is clinically appropriate. The house does not set a taper schedule or override clinical judgment.

This expectation is discussed before admission. If the resident or prescriber does not believe discontinuation during the program is appropriate, Spruce Street is not

the right fit. The house defines the program it is prepared to operate; the prescriber defines safe clinical care. Neither assumes the other's authority.

Testing, relapse and emergencies

Testing is observed, full-panel, random and scheduled, with laboratory confirmation of anomalous results. Observation is not a judgment about character. It ensures that a clean result is not left open to dispute.

Relapse does not result in automatic homelessness. Most often the response will involve detox with a possible return; sometimes a resident may remain in the house under significantly restricted status while the situation is assessed. Either way, he has somewhere safe to sleep that night. Nobody is put on the street for drinking or using. What follows depends on whether he wants to continue and whether his presence can be managed safely for everyone else.

Spruce Street maintains naloxone in the house and trains staff and residents to use it. Staff are awake and on site overnight. Relevant allergies and medical information are recorded at intake and immediately available. When a situation exceeds the house's competence, the first action is to call emergency services.

One rule outranks every concern about honesty or discipline: nobody is ever in trouble for calling for help — not the resident in difficulty and not the person who gets assistance. Hesitation about that call can be fatal, and the rule is never qualified after the fact.

When a resident's needs become clinical, acute or otherwise beyond what a non-clinical sober residence can safely hold, the house says so and helps him reach an appropriate level of care. That is not the model failing. It is the model being used honestly.

SECTION 7

Family, Clinicians and Long-Term Community

By the time a resident arrives, his family has often spent years doing a job nobody trained them for: tracking his location, financing emergencies, negotiating promises and lying awake doing the arithmetic on whether helping had become rescuing. That posture does not disappear because the immediate danger has moved.

Spruce Street gives families a steadier role. The house — not the family — manages daily expectations, money, transportation, freedom and consequences. A resident cannot route around the structure by calling home for extra cash, a softer answer

or rescue from a consequence he earned. The boundary lets him solve problems where they live and gives his family permission to discontinue a role that may have consumed the relationship.

Families are not left guessing. With a signed release, communication is generally weekly during the first months and as needed afterward. Updates cover current work, progress and genuine concerns. The aim is to explain the larger arc so that an uncomfortable week is not mistaken for a failing program — and to say plainly when the picture is concerning.

Every two weeks, families gather at the house for lunch and residents host. The men plan the meal, cook, set up and clean afterward, including those whose own families can't make it. The inversion matters. Many have spent years attending family gatherings as the person everyone was quietly worried about. Here they are responsible for putting on the afternoon.

The gatherings also let families know one another. A mother convinced that she is uniquely at fault may spend twenty minutes with someone further down the same road. That conversation often does more than anything staff could tell her.

The house does not promise reconciliation on a schedule. What it can build are conditions in which trust has somewhere to regrow: sustained follow-through, direct amends, an end to hidden negotiations and enough stability that conversations can return to life outside addiction.

Working with outside clinicians and referral partners

Spruce Street is intended to extend — not compete with — the work of treatment. With consent, the house can provide clinicians a second view of progress: whether a resident's stated insight holds in transportation, work, conflict, money and unstructured time. Communication should be useful and proportionate, not a stream of minor infractions.

The house wants the same candor in return. Referring professionals should tell us what they have seen, what has repeatedly preceded relapse, what clinical needs must remain outside the house, and what would make the placement unsafe or poorly matched. When the fit is wrong or a resident needs a different level of care, we will say so rather than defend the placement.

Because Spruce Street is still in development, early referral relationships will help shape written policies, clinical coordination and launch standards. We are circulating this overview now so people who know the population can identify what they need to see before entrusting someone to the house.

Alumni and the durable community

In a mature house, alumni are not ceremonial visitors. They are part of its daily mechanics. They sponsor residents, answer calls from the ride sheet, return for the weekly long group and show newer men what life after the program looks like.

An alumnus holds no authority and enforces nothing. He can be affectionate and unimpressed in the same sentence. He can tell a resident with six weeks that he also believed the structure was absurd — and be believed in a way staff may not be. He is proof that the future is real, standing in the kitchen.

The arrangement runs both ways. Graduates receive a way to be useful, a place to walk into on a bad Tuesday and a home for the friendships formed during the hardest year of their lives — friendships that, a decade on, are often still the closest they have. They arrived with housemates. They leave with brothers. Graduation is not an exit from the community but a change in one's place inside it.

This network cannot be manufactured or assigned as service hours. It will take years of graduates before it carries its own weight. Spruce Street's obligation is to begin honestly: keep the door open and the number working, make the relationships real and remain available after a resident's monthly payment and formal program have ended.

SECTION 8

Outcomes, Founders and Development Status

What will be measured

Spruce Street has no outcomes to report because it has no residents or graduates. The model makes claims that can be examined, but experience operating a related program does not entitle us to present expectations as Spruce Street's results.

If the structure works, residents should move through progressively less controlled phases, demonstrate reliability across work, money, relationships and independent movement, leave with stable housing and a credible plan, and remain sober, connected and productively occupied after the house no longer organizes their days.

From the first admission, Spruce Street intends to track:

- length of stay for every resident, including early departures;
- the phase reached and whether the resident graduates;

- destination and level of care after departure;
- sobriety, housing, employment or education, and engagement with recovery supports at twelve and twenty-four months; and
- the proportion of former residents actually reached at each interval.

Response rates will appear beside follow-up figures. **A success rate calculated only from graduates who still answer the phone is not a finding.** We would rather publish a smaller honest number than a larger unverifiable one.

Measurement should also be capable of correcting the program: which residents leave early and when, which admission judgments prove wrong, and which restrictions produce useful practice rather than simple compliance.

Founders and relevant experience

Ty Stillerman and Chase Wilson met nearly a decade ago as residents of New Life House, and both have remained active in the recovery community since.

Ty Stillerman is a co-founder of Spruce Street Recovery and will lead the organization's programming and daily operations. He spent four years as Assistant Program Director of New Life House in Los Angeles, the long-term recovery program on which much of Spruce Street's model is based. In that role, he helped oversee day-to-day house operations and worked closely with residents through the behavioral and interpersonal challenges of long-term recovery.

Ty earned a B.A. in Economics and Public Policy from the University of California, Berkeley. He now works at the Foundation for Individual Rights and Expression, where much of his work involves getting people who disagree profoundly to talk to each other rather than at each other—which is, as it happens, much of what running a house like this consists of.

His experience as both a resident and an Assistant Program Director informs Spruce Street's approach to building a structured, accountable community in which men learn to live differently over time.

Chase Wilson is a co-founder of Spruce Street Recovery and will lead the organization's business operations and finance. He spent three years as a Program Manager at The People Concern in Los Angeles, where he led street outreach efforts connecting people experiencing homelessness with treatment, housing, and other supportive services.

Chase earned a Master's in Urban Planning from the University of Southern California, where his work focused in part on public health and the economic impacts of the opioid epidemic. He is currently completing his MBA at the USC Marshall School of Business, with an expected graduation in December 2026. He is also a co-owner of

Cafe Tropical, a longstanding Cuban restaurant in Los Angeles that he and a group of partners purchased and reopened. Cafe Tropical has long served as a gathering place for the local recovery community, hosting more than 30 twelve-step meetings each week.

Chase's background in social services, urban policy, and business is aimed at keeping Spruce Street operationally durable as well as mission-driven.

Their biographies do not prove the model. They explain why its sequence matters to them. Both spent years reaching for the visible parts of adult life before building the ordinary conduct that could hold those lives up. Both were carried by a community before becoming useful inside one. Spruce Street is an effort to reproduce that sequence deliberately, preserve what was transformative and draw firmer lines around what should not be repeated.

Development status

The details are still taking shape. The model is not. The program is expected to open by the end of 2027. Its length, population, phased structure, accountability system, Twelve Step foundation, private-pay approach, non-clinical scope and opioid-maintenance boundary are settled.

In progress are the property and municipal approvals, final capacity and fee, permanent name, certification timing, deposit and refund terms, written admissions and medication policies, launch staffing schedules and ratios, and an advisory circle of clinicians, operators and people in long-term recovery.

The largest first-year limitation is structural: a model that relies on senior residents opens with none. Staff and carefully selected people from the recovery community will perform functions that a mature house distributes. Responsibility will not be handed out to make the phase system appear complete; peer roles will expand only as residents acquire the experience and trust to carry them.

The alumni network begins with the same limitation. Staff and members of the broader recovery community will initially provide more transportation and presence. Each connected graduate will add one strand to a network that takes years to mature.

Resident responsibility is never a substitute for staff authority, awake coverage, emergency response or outside clinical care. Its purpose is that reciprocal obligation changes both men involved.

We are sharing this overview before opening because we want to build toward the actual needs and preferences of people who refer and treat this population.

SECTION 9

Appendix: Program Facts

The items below reflect the current model. Details identified as expected or anticipated remain subject to the property, municipal approval, final staffing design and legal review.

POPULATION	Men ages 24–35
LOCATION	Philadelphia; property search underway

CAPACITY	Roughly 18 beds. Final capacity depends on the property and municipal approval
LENGTH	Approximately 12 months to graduation; 12–14 months including in-house graduation. Initial commitment: 90 days
FRAMEWORK	Twelve Step, required; all twelve Steps completed with a sponsor before graduation
STAFFING	Current plan: Director, weekday manager, weekend manager and two night managers, with awake staff on site around the clock. Final staffing and ratios depend on the property and bed count
MOVEMENT	No resident leaves alone or unaccounted for except for work, beginning around month six. Departures and returns are logged. Independent movement returns during in-house graduation
TECHNOLOGY	No personal smartphone until in-house graduation. Early calls use a house line with a senior resident present. From Standing, residents may use a flip phone outside the house and for work. Monitored house computers are available for approved purposes. Smartphones and laptops return during in-house graduation
MONEY	House-held on an internal ledger; equalized weekly allowance, daily written requests, signed disbursements and weekly reconciliation. Control loosens as independence returns
VEHICLES	Initially, house vehicles driven by staff and supplemented by recovery-community support. Residents may bring cars in the senior months with appropriate insurance; keys remain with staff overnight
EMPLOYMENT	Begins around month six, based on progress. Residents find their own jobs. The usual target is 30–35 hours weekly unless finances require more
TESTING	Observed, full-panel, random and scheduled, with laboratory confirmation of anomalous results
CLINICAL CARE	Therapy, psychiatry, medical care and detox are provided externally, arranged and paid for separately by the resident or family

MEDICATION	Prescribed and managed externally; stored in a locked staff office and self-administered with staff present. No in-house prescribing or adjustment
OPIOID-MAINTENANCE MEDICATION	Residents may enter on maintenance medication, with the expectation that they complete the program without it. The outside prescriber controls the pace and clinical process of any taper
FAMILY COMMUNICATION	Generally weekly during the first months and as needed afterward, on a signed release. Family gatherings occur at the house every two weeks and are hosted by residents
ADMISSIONS PROCESS	Director-led assessment of addiction history, behavioral fit, informed willingness, safety, medication and clinical needs; consultation with the referring professional or program; and family interview
ADMISSION LIMITS	Declined or deferred for acute withdrawal, medical instability, psychiatric symptoms that cannot be managed safely in a non-clinical residence, active risk of harm, or certain strong recent histories of violence
COURT INVOLVEMENT	Probation and parole involvement accepted where admission remains voluntary, with required reporting. Court-ordered residence at Spruce Street is not accepted
COST	Private pay; expected range of **$5,000–$7,000 per month**. One inclusive fee with no add-on billing for the program's testing, groups or transportation. Final price and inclusion of food and allowance depend on property and staffing. Outside clinical services are separate
TERMS	Anticipated first- and last-month deposit with prorated refund provisions; being finalized with counsel
CERTIFICATION	Spruce Street intends to pursue PARR/NARR certification

REFERRAL CONVERSATIONS & FEEDBACK

We welcome direct questions, criticism and referral-partner input.

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This document describes a program in development and is not an offer of services.

